

## Written Record of the Care to be Provided to Resident

The California Health and Safety Code 1569.80 and California Code of Regulations, Title 22 section 87467, requires all residential care facilities for the elderly to have the resident or resident's representative and other interested persons participate in creating a written record of the care and services the resident will receive in the facility and the resident's preferences regarding the services facility provides. This contract of admission serves as a written record of that provision of care. The items listed below are to be included in the written record:

1. The resident's preferences regarding facility services.
2. Date of the meeting.
3. Names of individuals who participated in the meeting.
4. Their relationship to the resident.
5. The agreed-upon resident services to be provided.
6. Absentee notification.
7. A copy of the record sent to the resident's regular physician; and
8. To review and, if necessary, revise the written record when there is a significant change in the resident's condition, or once every 12 months, whichever occurs first.

The date this written record was formulated \_\_\_\_\_

Signed by:

Family User

FC935CDD3BA4465...

The names and relationships of those participating in this meeting were:

Name: \_\_\_\_\_ Relationship: \_\_\_\_\_

Name: \_\_\_\_\_ Relationship: \_\_\_\_\_

Name: \_\_\_\_\_ Relationship: \_\_\_\_\_

Name: \_\_\_\_\_ Relationship: \_\_\_\_\_

Name: \_\_\_\_\_ Relationship: \_\_\_\_\_

There are numerous services available in this contract of admission that includes basic services, optional services, and third-party services. The resident and/or resident's representative will indicate which services are those desired by, needed by, and that will be provided to the resident by initialing a line ( \_\_\_\_\_ ) on subsequent pages.

Facility reserves the right to review this written record at any time to determine that the facility's currently provided services meets the resident's current needs for services. This will involve a reevaluation (reappraisal) of resident needs, and a meeting with the individuals listed above. A physician's evaluation (medical assessment) will also be included.

Signed by:

Facility User

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Resident/Representative Initials \_\_\_\_\_ Facility Representative Initials \_\_\_\_\_

02/06/2022

## Admission Agreement and Contract

### General Provisions

In this agreement the resident will be referred to as “resident;” whereas resident family members, resident’s representative, and/or conservator will be referred to as “resident’s representative” in this agreement. Facility or licensee, as written in this agreement, is the operator of the facility and referred to as “facility” or “licensee.” Facility may also include employees that render services for and on behalf of facility or licensee.

The following is facility’s Admission Agreement (“agreement” or “contract” or “contract of admission”) and is a valid offer of services. This document and all documents attached or presented at the time of admission are required to be signed at the time of and condition of admission. A signature to this contract represents a mutual acceptance of the provisions contained in this contract and is legally binding upon both parties to perform its portion of the contract. This contract binds both parties to the terms and conditions contained within. Failure of either party to adhere to the terms and conditions may constitute breach of contract. In addition, no rights or duties contained within this contract can be assigned or transferred or delegated to other persons or parties.

A residential care facility for the elderly, licensed by the California Department of Social Services (Department) is not allowed to provide 24-hour skilled nursing care. This facility does not and cannot provide “one-on-one care.” *The facility does not assign one caregiver to provide 24-hour escort, bedside care, or continuous observation.* Under California Health & Safety Code 1569.1, this licensed facility is designed to provide a “humane approach to meeting the housing, social and services needs of older persons,” and to “provide a homelike environment for older persons with a variety of care needs.”

### Complete Contract

This document is the entire agreement and any discussions and/or oral or implied terms which are not included in this contract are excluded and are not considered to be part of this agreement. This is the entire agreement between both parties. Modifications and amendments to this agreement, including any exhibit or appendix, shall be enforceable only if they are in writing and are signed by authorized representatives of both parties.

### Admission Policies

This facility does not discriminate against a person seeking admission based on sex, race, color, religion, national origin, marital status, registered domestic partner status, ancestry, actual or perceived sexual orientation, or actual or perceived gender identity.

Gender Identity means a person’s stated identification with a sex, regardless of whether the stated sex is consistent with the person’s physical appearance, surgical history, genitalia, legal sex, sex assigned at birth, or name and sex, as they appear in medical records, and regardless of any statement by any other person, including a family member, conservator, or legal representative, that is contrary. A person who lacks the

ability to communicate gender identity retains the gender identity most recently expressed by that person.

Because of recently passed legislation, the facility is compelled to implement record-keeping procedures to ensure it ascertains the client's gender identity, correct name as indicated by the resident, and the use of the correct pronoun (he, she, his, her, etc.) when referring to the resident. If resident wishes to decline to provide the information, that is a resident's right to do so. However, because facility can be cited for not attempting to obtain such information, client is encouraged to provide:

Client's gender identity \_\_\_\_\_

Client's correct name \_\_\_\_\_

Client's correct pronoun \_\_\_\_\_

☐ I choose to exercise my rights and refuse to disclose the above.

\_\_\_\_\_  
Resident or Resident's Representative

### COVID-19

The Department requires all residential care facilities to obtain a negative Covid-19 test prior to any resident admission, or resident can present proof of being "fully vaccinated." The Department defines "fully vaccinated" as two (2) weeks after receiving the second dose in a 2-dose series, like the Pfizer or Moderna vaccines, or two (2) weeks after a single dose vaccine, like Johnson & Johnson's Janssen vaccine.

Resident will be further screened for Covid-19 symptoms i.e., fever or chills, cough, shortness of breath or difficulty breathing, fatigue, muscle or body aches, headache, new loss of taste or smell, sore throat, congestion or runny nose, nausea or vomiting, diarrhea, trouble breathing, persistent pain or pressure in the chest, new confusion, inability to wake or stay awake, pale, gray, or blue-colored skin, lips, or nail beds, depending on skin tone, and the resident has not had contact with anyone with a confirmed case of Covid-19 in the past 14 days.

If some of these symptoms are present, the facility has the right to delay admission until symptoms are cleared, or to admit, but place resident in isolation for up to 14 days, which would be in a private room, if available, and charged at facility's private room rate. Facility does not guarantee it can admit a symptomatic resident if a private room is not available or symptoms are judged to be severe.

### Resident Language(s) Read

Under a recent revision to the California Code of Regulations, the Department may seek from the facility "the language(s) read by each resident. DSS claims this information will be kept confidential. Please indicate the language(s) read by the resident:

English \_\_\_\_\_

**Referral Agencies**

This facility may work with public or private referral or resident placement agencies. This facility, if it accepts such a referral and accepts and retains a resident referred by one or more referral agencies, must pay a referral or finder's fee to such agencies. If you have previously contacted an agency to assist you in finding a residential care facility for placement of the resident, please list here:

\_\_\_\_\_ Date of initial contact \_\_\_\_\_  
\_\_\_\_\_ Date of initial contact \_\_\_\_\_

**Placement History**

Facility anticipates the ability to meet all of our residents' needs as identified in the physician's medical assessment and facility's initial and ongoing appraisals. Facility asks the resident or resident's representative to disclose past facility placements or admissions in other residential care facilities and the reason(s) for leaving, as facility wants to ensure this will be the last facility the resident will need. Resident may refuse to disclose this information, but in order to accommodate all resident service needs and to prevent a future move, the facility asks resident or resident's representative to disclose:

☒ I have not previously been admitted into or placed into residential care.

☐ I have been in other residential care facilities and the reason(s) for moving or relocation is/was \_\_\_\_\_  
\_\_\_\_\_

**Modification Conditions**

Facility may, with a 30-day written notice to resident or resident's representative, amend this contract when such an amendment does not involve fees. Resident or resident's representative will be asked to sign and date the amendment. In addition, facility will provide resident and/or resident's representative with a 30-day written advanced notice of a necessary room change including an emergency or to fill a vacant bed. Resident may agree to waive the 30-day notice and move immediately to a different room.

**(See Attached Addendum to Admissions Agreement "Roommate Change Policy" Rate Increases**

The facility reserves the right to increase the basic monthly fee, its rate structure for services including its fees for its levels of care by providing a 60-day written notice. Rate increases will not exceed 5% of basic monthly fee in a 2-Year period. The notice sent to the resident and resident's representative will plainly specify the reasons for the increase and a general description of the factors for the increase. Any increase due to a level of care or condition change may be implemented after providing a two-business day written notice.

Resident/Representative Initials \_\_\_\_\_ Facility Representative Initials \_\_\_\_\_

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**Absentee Notification**

Under California Health & Safety Code 1569.317, every residential care facility for the elderly shall develop and comply with an absentee notification plan to address issues that arise when a resident is missing from the facility. The plan shall include a requirement that an administrator of the facility, or designee, inform the resident's representative if/when that resident is missing from the facility and the circumstances in which an administrator of the facility or designee, shall notify local law enforcement when a resident is missing from the facility. The circumstances under which the facility's administrator or designee will notify local law enforcement is immediately upon the discovery that the resident cannot be found to be in the facility or on the facility grounds. If resident is missing from the facility the contact person is

\_\_\_\_\_ to be reached at \_\_\_\_\_

**End of Life Option Act**

Facility has elected, for reasons of conscience, morality or ethics, to not have its employees participate in activities related to the "end-of-life" act. This includes delivering the prescription for, dispensing, or delivering the prescribed aid-in-dying drug; and being present when the resident takes the aid-in-dying drug prescribed by resident's attending physician. Any decision made by the qualified individual to obtain and/or take aid-in-dying medication will NOT result in resident eviction or threat of eviction, discrimination, or harassment. Facility will NOT place aid-in-dying medication in its centrally stored medication location, and if resident stores the medication in his or her room, resident's physician must indicate the client is capable of self-storage of personal medications. Facility does require this medication to be locked in a safe place within the resident's personal storage space. The resident is NOT required to disclose to any facility personnel the intent to use the aid-in-dying medication, but the enacted law does require a witness to the resident self-administering this medication. Facility staff will not act as a witness due to reasons of conscience, morality and/or ethics. Facility staff does not witness any verbal or written notices presented to a physician about the desire to obtain and/or use aid-in-dying medications.

**Hospice**

If resident is recommended by a physician to be placed on hospice for a terminal illness as certified by two physicians, facility will adhere to Title 22 sections 87632 and 87633 and ask resident and/or resident's representative to submit a written request to facility to be placed on hospice while residing in the facility. Facility must approve the written request for retention and approve all hospice care plans prior to the initiation of hospice services by an outside agency. The facility has worked with many hospice agencies in the community and may refer or recommend resident to agencies trusted by the facility. Facility reserves the right to reject the hospice agency selected by the resident or resident's representative if facility has had trouble with that agency in the past. The facility requires resident and/or resident's representative to inform facility when client

may be placed on hospice, so all the requirements of the state can be met, and facility may evaluate the services hospice agency claims to provide.

### **Assessment and Evaluation of Resident**

The facility must, as required by state regulation, conduct a preadmission appraisal of resident needs and services and the facility's ability to meet those needs. This appraisal is conducted by the administrator or designee using the criteria found in state regulation sections 87458, 87459, 87461 and 87462. Reappraisals are conducted when there is a significant change in the resident's condition according to the provisions found in state regulation section 87463, and annually if the resident has a physician diagnosis of dementia. This reappraisal involves the resident's physician, family and facility staff, and may result in initiating a level of care increase (see Levels of Care).

### **Care Plans/Needs and Services Plans**

This facility is licensed by the California Department of Social Services and is not a medical entity. Facility has no requirement to author or use "care plans," "plans of care," "service plans," or "needs and services plans." Such plans are for hospitals, nursing homes, home health agencies, hospice agencies, etc. as care plans are a medical approach to care. Under Health and Safety Code 1569.1, residential care facilities are "not primarily medically oriented" and are a "homelike," "social" setting. Should you or a payor source require care plans, facility will provide this admission agreement to you or to a payor source, with resident consent, that outlines all required and provided care, including basic services and levels of care.

The Department of Social Services has no law or regulation requiring care plans as that would classify this facility as a medical provider. If you access home health or hospice, these agencies are required under state laws—Health and Safety Codes 1569.725 and 1569.73, respectively—to author care plans. Facility is charged with reading, approving and implementing the plans, but must also operate within the confines of a social model of care. Any medical care provided by outside medical agencies cannot be evaluated by facility, and facility is not responsible for providing medical care that is outside the confines of allowed care.

### **Failure to Provide Complete Information**

Facility is required by state law and regulation to conduct a preadmission appraisal and any needed reappraisal to determine the care to be provided to the resident. Facility and resident, and/or resident's representative, must be forthright in providing all needed and necessary information to determine suitability for admission and retention in residential care. Any false or misleading information provided by the resident or resident's representative may void this contract, according to state contract law, and lead to relocation or eviction.

### **Change of Licensee**

In the event of a change of facility ownership or transfer of property or business, facility will provide at least a 60-days written notice to resident, resident's representative, and Community Care Licensing of the facility's intent to sell.

**Facility Information**

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**Licensee Information**

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Facility # \_\_\_\_\_

**Resident Information**

Name of Resident \_\_\_\_\_

Social Security Number (optional) \_\_\_\_\_

Date of Birth \_\_\_\_\_ Date of Admission \_\_\_\_\_

**Resident's Representative Information**

Resident's Representative (if not resident) \_\_\_\_\_

Relationship \_\_\_\_\_

Address \_\_\_\_\_

Phone Number \_\_\_\_\_

**Acknowledgement of Average Monthly Rate Increases (H&SC 1569.658)**

California law requires that this facility, at the time of admission only, disclose to the resident and resident's representative its average monthly rate increases for living units and service fees, the average amount of the increase, and the average percentage of increase for each of the previous 3 years. (If facility has been in operation less than three years it will disclose any rate increases over its operating history.) This disclosure is updated by January 31 of each year. If you have received this form at the time of admission, please acknowledge its receipt. This will be kept in the resident file.

\_\_\_\_\_  
 Resident or Resident's Representative

**Basic Services**

As noted above, these basic services represent the facility's "plan of care," "care plan," or "service plan." To indicate acceptance of the services below, please initial each line \_\_\_\_\_. The facility will assure the provision of these basic services:

Resident/Representative Initials \_\_\_\_\_ Facility Representative Initials \_\_\_\_\_

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1. Care and supervision as defined in Health and Safety Code 1569.2; One-on-one care is not provided.
2. Monitoring and observing for changes in physical, mental, emotional and social functioning.
3. Notification to resident's family, physician, and other appropriate person/ agency of resident's needs.
4. Lodging: X Private Room N/A Shared Room. Facility's room rates begin at \$ N/A , which represents a shared room at the basic services rate or Level One. (This rate represents a resident that is alert, self-care, ambulatory, has full continence, emotionally stable, and no special dietary needs.) If admitted into a shared room the facility will adhere to California Health and Safety Code 1569.269(a)(19) allowing a resident: "To share a room with the resident's spouse, domestic partner, or a person of resident's choice when both spouses, partners, or residents live in the same facility and consent to the arrangement." In addition, Health and Safety Code 1569.269(a)(17): "To reasonable accommodation of resident preferences concerning room and roommate choices." Facility will make every attempt to provide a compatible roommate but cannot accommodate all residents with roommates. In this case a private room may be more suitable. A private room, when available, is charged at an additional rate above the double room rate. This additional rate is \$ N/A . Initialing here N/A acknowledges this optional charge.
5. Food Services:
  - a. Three (3) nutritious meals per day and between meal snacks.
  - b. Special diets if physician prescribed
6. Helping gain access to supportive services, which may include medical, dental, and other health care services; transportation; recreational and leisure activities; and social and counseling services.
7. Plan, arrange and/or provide for transportation to medical and dental appointments. Facility will assist with making arrangements for transportation to and from medical and dental services.
8. A planned activity program, including arrangement for utilization of available community resources as follows: libraries; community centers; and senior centers. A list of these is available from the facility.
9. Assistance with personal activities of daily living as follows:
  - a. Dressing.
  - b. Eating (including feeding assistance).
  - c. Toileting.
  - d. Bathing.
  - e. Assistance with taking prescribed and over-the-counter medications in accordance with physician's instructions unless prohibited by law or regulation.
  - f. Grooming.
  - g. Mobility; and



- h. Other personal care needs: housework, meals, laundry, taking of medication, appropriate transportation, correspondence, telephoning, and related tasks.
10. Basic hygiene items such as general use soap and toilet paper. Other personal toilet articles, i.e., toothpaste, Kleenex, mouthwash, shampoo, etc. are the responsibility of the resident or resident's representative.
  11. Safe, clean, sanitary and healthful living accommodations.
  12. Laundering of personal clothing. General laundry service (washing and drying of personal laundry [excluding dry cleaning]).
  13. Clean bed and bath linens weekly, or as often as needed.
  14. Cleaning of resident's room at least weekly.
  15. Comfortable and suitable bed and bedroom furniture. Facility no longer allows resident bed or bedding to be brought into the facility due to risk of infestation of bedbugs, scabies, etc. However, if resident or resident's representative wishes to bring in bed and bedding or any other items facility believes may be infested, facility does require proof of fumigation or treatment of said items. Facility regrets this requirement but believes the protection of its residents and staff is the highest priority for care and liability.
  16. Assistance in meeting necessary medical and dental needs as follows: Facility encourages resident's representative to transport resident to and from medical and dental appointments. Facility will transport resident as needed to appointments if resident's representative is unavailable. Charges will apply.
  17. Bedside care and tray service for minor temporary illnesses or recovery from surgery. Tray service during the temporary illness or recovery from surgery is provided without charge for two days. After two days, a fee will apply.
  18. Facility provides access to facility phone for the purposes of making local phone calls and receiving phone calls. Facility encourages resident to use a prepaid phone card or cell phone for long distance calls. If resident lacks the mental and/or physical capacity to dial phone, facility will assist.
  19. To practice universal precautions, facility staff will use gloves when caring for all residents. Gloves are charged to the resident at facility cost, and each resident's bedside will have a box of gloves available for staff and resident.

### Health Conditions

This facility does not and cannot provide medical care and is licensed by the Department to meet resident's need for care. Clients that need emergency or hospital inpatient care or skilled nursing care may not be allowed to return to the facility if the facility assesses a resident and documents the client has a medical need that cannot be cared for within the limits of its license. The facility will advise resident's physician and resident's representative of the inability to meet client needs in addition to seeking placement at a higher level of care, if and until, the resident qualifies for residential care.

### Blood Pressure/Heart Rate Monitoring

If resident requires blood pressure or heart medications based upon vital sign readings resident must provide facility with equipment necessary for staff or resident to take vital sign readings, such as a blood pressure machine. Facility requests that any equipment furnished for such readings be in good working order. Resident may take own vital sign readings upon verification from resident's physician of resident ability to do so.

### Diabetics

Facility may and can accept diabetic residents. However, facility staff is unable, by state regulation, to administer injectable medications including insulin. Resident's physician must verify resident can "self-administer" injectable medications, and facility reserves the right to have physician verify resident's ongoing ability to self-administer injectable medication. Should residents using or needing injectable medication lose the ability to self-administer injectable medication, it constitutes reason for relocation via eviction as facility would not be able to meet the resident's need.

### Levels of Care

Any increase due to a level of care or condition change may be implemented after providing a two-business day written notice, which is allowed under California Health and Safety Code 1569.657. This includes a mandate that the facility provide a "detailed explanation" of the additional care provisions included in each level. Each level of care is "resident-specific" and uses a "point system" to objectively evaluate the resident. Facility uses the RCFE Levels of Care Assessment Tool.

The levels of care were implemented by the California State Legislature. *A change in the resident's condition is documented by the resident's physician using the state-standardized physician's report form—LIC602A—and by the facility using the state-standardized form LIC603A—prior to implementation of the levels of care.*

N/A Level 2—Minimal Care and Supervision. Residents at this level present with mild confusion, some incontinence, medically stable but with mild emotional or behavior issues. Facility will provide the following additional services:

- Verbal reminders to perform hygiene functions.
- Monitor dressing, grooming, bathing and personal hygiene.
- Assistance in use of assistive devices such as a walker or wheelchair.
- Incontinence care, with verbal reminders to toilet, occasional brief changes.
- Redirection for behavior issues.
- Medically necessary diets.
- Monitor weight including monthly weights.

Additional fee of \$ 0 is in addition to Basic Services Fee.

X Level 3—Moderate Care and Supervision. Residents at this level present with moderate confusion, are nonambulatory, use assistive devices, has a higher degree of incontinence, and has an allowable/restricted condition. Facility will provide the following additional services:

- Services in Level 2.

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- Verbal reminders to time and place with recent events.
- Direct and assist with basic hygiene maintenance.
- Assistance with transferring to walker or wheelchair with staff.
- Incontinence care, including hygiene, infrequent brief changes, nighttime checks, including more cueing to toilet.
- Assist with “as needed” medications for behavior behavioral issues in addition to redirection.
- Assist with accessing home health for allowable/restricted health conditions, as required by Health and Safety Code 1569.39.
- Provide active intervention in weight management, including weekly weight monitoring.

Additional fee of \$ 0 is in addition to Basic Services Fee.

N/A Level 4—Extensive Care and Assistance including Allowable Health Conditions. Residents at this level present with disorientation, unstable mood and behavior due to dementia, inability to perform activities of daily living, total transfer dependence, complete incontinence, and possibly bedridden. Facility will provide the following additional services:

- Services at Levels 2 and 3.
- Verbal and visual reminders to time, place, meals, grooming, activities of daily living.
- Monitor and prevent wandering via active staff intervention with redirection.
- Two-staff transfer assistance.
- Assist with use of mechanical devices, i.e., Hoyer lift.
- Incontinence care, with frequent changes of briefs, possible showering, and nighttime changing.
- Assist with care planning with home health.
- Feeding assistance with all meals and snacks.
- Provide assistance with nutritive supplements, i.e., Ensure, vitamins.

Additional fee of \$ 300 is in addition to Basic Services Fee.

N/A Level 5—Full Assistance and Possible Restricted/Prohibited Health Conditions, possible exception may be required by the Department. Residents at this level present with severe dementia, confusion, sundowning, elopement risk; qualify as “total care” assistance with most activities of daily living; uncooperative and/or combative; bedridden; hospice care; and obesity issues. Facility will provide the following additional services:

- Services at Levels 2 – 4.
- Active intervention in sundowning avoidance.
- Two-caregiver assistance with transfers, bathing and hygiene required for this higher level of care.
- Monitor elopement risk.
- Assist in participating in activities to reduce the use of psychotropic medications.

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- Turn and/or reposition in bed if bedridden every two hours or as prescribed by hospice or home health.
- Assist hospice agency in creating and adhering to care plans.
- Assist in weight management via special diets or prescribed exercises.

Additional fee of \$ 300 is in addition to Basic Services Fee.

Any additional services not contained in this agreement will be given to and reviewed with the resident and/or resident's representative and will only be implemented upon the signed and dated acknowledgement of said parties.

### Funding Source

The resident has the right to refuse to disclose the monthly fee-funding source. To decline to disclose this information simply write, "refuse to disclose" in the blank below. My funding source for payment of the monthly fees is \_\_\_\_\_. The Department recommends that residents voluntarily disclose in this admission agreement whether the rate paid to the facility includes SSI/SSP benefits. If the resident is an SSI/SSP recipient, please indicate below.

The resident funding source \_\_\_\_\_ does include SSI/SSP funds  
 \_\_\_\_\_ does not include SSI/SSP funds

*The facility **does not accept SSI/SSP-dependent residents** and cannot accept SSI/SSP-issued Personal and Incidental Needs monies as higher payment for services. Facility does not accept voluntary contributions from others as payment. Failure to reveal to the facility the SSI status of the resident may void this contract.*

### Optional Services

Optional services are those services provided by the facility which have additional fees, require a resident's initials \_\_\_\_\_ on each provision, and are listed below:

\_\_\_\_\_ Bathing—Facility provides the resident with two baths/showers per week. Additional baths are an optional service and are charged at a rate of \$ 15 per bath. If accepted, the number of additional baths per week is 1.

\_\_\_\_\_ Long Distance Phone Charges—If resident uses facility phone to make long distance phone calls, these calls are charged to the resident at the cost of the call. A copy of charges will be placed in the resident's file and itemized on a monthly statement. Resident may install a private line in resident living area at resident expense. Resident or resident's representative must pay all charges associated with installation and maintenance of the phone and phone line and pay all charges for usage.

\_\_\_\_\_ Special Food Services—Tray service, a special food service, is charged after two days if the resident is unable, not unwilling, to come to the dining room. After two

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days, a fee of \$ 5 per tray will be charged. This charge will be itemized at the end of the month. Residents who are unwilling to come to the dining room are not provided two days of tray service without charge.

\_\_\_\_\_ Special Food Products—Special food items are those food items not ordinarily used by the facility and are purchased for an individual resident's use and consumption at the request of the resident. This includes diabetic foods. If requested, special food products purchased for an individual resident will be itemized on a monthly statement and charged at facility's cost [as allowed by Title 22 section 87464(e)(3)].

\_\_\_\_\_ Ironing of Clothing—Facility will iron resident personal clothing for a fee of \$ 5 per piece.

\_\_\_\_\_ Transportation—Resident's representative may and are encouraged to provide basic transportation. Facility may provide resident with transportation when resident's representative or an alternative arrangement is unavailable, to be billed at a minimum of \$ 50 per hour or any portion of an hour to cover driver and/or caregiver escort expenses. All transportation arrangements require a minimum of a 48-hours' notice. Facility will make alternative arrangements if family is unavailable or does not wish to pay facility's hourly charges. This may include using local agencies that specialize in resident transportation.

\_\_\_\_\_ Private Laundry Service—Private laundry service is charged at \$ 5 per load. If this service is requested, resident laundry is not combined with another residents' laundry. Resident's representative may do resident laundry to avoid any additional charges.

\_\_\_\_\_ Medication Management—The facility assists with self-administered medication. All medication must be prescribed and authorized by the resident's physician. Medication management is charged at \$ 0 per month and involves the use of a uniform medication packaging system, and the following assistance:

1. Reminding the resident to take medication.
2. Check all delivered medications for correctness of medication and dosage.
3. Observing the resident taking all medication.
4. Assisting with self-administration; and
5. Ordering and reordering medication.

### **Breakage/Damage Fee**

The facility is not allowed to collect a deposit against possible damages caused by the resident. There is no prohibition in Title 22 or Health and Safety Code for charging the resident or resident's representative for damages that may be caused by the resident. This includes breakage of facility property, blocking of toilets or plumbing by placing inappropriate items into plumbing fixtures (i.e. clothing, adult briefs, etc.), and the soiling of facility carpeting. The facility will not attempt to collect a fee for normal wear and tear

caused by the resident, and any suspected damage or breakage will be discussed with resident or resident's representative. Any amount billed for damages will be the actual cost of replacement or repair.

### Payment Provisions/Rate Increases

The fee for the basic, optional and additional levels of care services must be paid in advance beginning on the first of the month and due the first of every month thereafter. If the basic monthly fee is not received within three days of the due date, a late fee of \$ 25 per day will be assessed for each day that the fee is not received. Payment may be made by check or cash. Should the facility receive a returned check from the bank, there will be a \$ 35 charge to cover bank fees and special handling costs, plus \$ 25 per day until secured payment is obtained. If the fee is not received within 10 days of its due date, eviction procedures will begin. The facility reserves the right to increase the basic monthly fee and fees for its levels of care by providing a 60-day written notice. The notice sent to the resident and resident's representative will plainly specify the reasons for the increase and a general description of the factors for the increase.

### Total Monthly Fees

|                        |          |
|------------------------|----------|
| The Basic Services fee | \$ _____ |
| Level(s) of Care fees  | \$ _____ |
| Optional Services fee  | \$ _____ |
| Private Room Fee       | \$ _____ |
| Total of               | \$ _____ |

The facility accepts cash, personal check, bank draft, or money order. Payment is to be made directly to the Licensee, payable to \_\_\_\_\_ at the address shown on page seven of this agreement.

### Third-Party Services

Third-party services are those not operated, provided or controlled by the facility. The fees will vary depending upon provider. The resident or resident's representative will enter into a contract directly with the third-party services provider. Third-party services include, but are not limited to

1. Podiatry,
2. Pharmacy services,
3. Cosmetology services, or
4. Private Companion.

Any fees charged by a third-party service provider will be posted in the facility and are to be paid directly to the third-party service provider. These services may be arranged in and by the facility when requested by the resident or resident's representative. Facility reserves the right to monitor and assess the quality of these services. Rates for third-party services may increase without notice, and facility is not responsible for advising resident or resident's representative of increases. Some third-party providers, if hired by



the resident or resident's representative, may be subject to the conditions listed under "Private Companion."

**Renter's Insurance**

This facility does NOT charge rent or refer to its service fees as "rent." This facility is NOT governed by Fair Employment and Housing. California Health and Safety Code 1569.147 states facilities are not under any form of rent control. However, facility strongly recommends that resident obtain "renter's insurance" that can be obtained through most insurance companies to protect resident's clothes, electronics, furniture, prosthetic devices, etc., should these be lost or damaged due to fire, earthquake or other "natural disaster." Facility's insurance does not cover resident's personal belongings.

**Hoarding**

Hoarding is defined as "the persistent difficulty discarding or parting with possessions, regardless of their actual value." Facility has specific fire clearance requirements and is unable to allow residents to "hoard" items that may block exits and passageways, or to take up space used for necessary personal items. Resident is not allowed to hoard disposable items such as newspapers, magazines, shopping bags or anything else determined by the facility administrator or staff to be a fire, tripping or other hazard. Facility does not "store" resident items that are not related to resident personal care or necessary for resident physical needs. Facility will discuss this issue with resident and/or representative.

**Tips and Loans**

The facility does not allow resident and/or resident's representative to tip facility staff or to loan money to staff or the licensee. Tipping and loans create numerous legal entanglements and paperwork for the licensee. It is clearly the policy of the facility to NOT allow tips or loans to be given to anyone affiliate with the facility, especially care staff.

**Resident's Use of Pharmacy Outside Facility's Uniform Packaging System**

Under California law a facility cannot require a resident "to purchase medications from any particular pharmacy." However, this same law allows a facility to establish reasonable policies and procedures that are necessary to assist residents with the taking of their medications. The facility's uniform packaging system, also known as "bubble packing," is a convenient and safer mechanism for the facility to manage a resident's medications. It involves specialized packaging used by pharmacies to reduce medication errors and ensures a reasonable assistance and assists facility in staying in compliance with state and federal laws and regulations. If resident does not wish to participate in the facility's medication uniform packaging system, facility reserves the right to charge resident \$ 35 per month. This charge is necessary as the facility must dedicate more time and efforts to safeguard medications, order medications, contact pharmacies, count medications, handwrite centrally stored sheets as required by the state, etc. Signing below acknowledges this extra charge IF the resident or resident's representative opts out of facility's Uniform Packaging System.



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Resident or Resident's Representative**Failure to Provide Resident Medication**

If resident's representative does not purchase physician-prescribed medication for facility to assist in medication administration, facility is not responsible for obtaining or paying for resident medication as allowed and mandated under California and United States contract law provisions known as the Statute of Frauds. Facility is required by law to report the failure to provide medication to the physician, law enforcement, Adult Protective Services, the Ombudsman, and the Department of Social Services. The failure of a resident's representative to provide medication may be prosecuted as elder abuse, a criminal offense in California. Facility is not legally required to obtain the medication as this would be in violation of Title 22 "Health Care Surrogate Decision Maker" provisions.

**Crushing Medication**

Should resident require medications to be crushed or altered (i.e., cutting), by signing below, resident or resident's representative consents to allow facility to place medication(s) into food or other substances—called "hiding or camouflaging" by state regulation—to enhance taste or allow ease of swallowing. Facility will consult with pharmacy or resident physician to ensure that resident medications may be safely crushed or altered. This consent is required under Title 22, section 87465.

Because facility is not a medical entity, it may refuse to crush or alter medications if it believes it is not safe to do so or cannot comply with current state laws and regulations. Facility would prefer not being placed in a position of having to crush or alter medication and will review each situation on a case-by-case basis. Alternatives to crushing will be discussed with resident physician and/or pharmacy.

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Resident or Resident's Representative**Use of Medical Marijuana**

The facility does not allow the use of medical marijuana delivered via cigarette or other method of smoking or inhaling. Other forms of marijuana delivery may be allowed and must be discussed with the licensee prior to admission or prior to its usage. The facility does not allow the cultivation or growing of marijuana in the facility.

**Private Companion**

It is a resident right to have a private companion or caregiver according to the terms and conditions set forth in the admission agreement or contract. If resident or resident's representative hires a private companion, facility reserves the right to screen individual for a criminal record and tuberculosis at resident or resident's representative expense. Facility requires that a private companion be contracted through a home health or home care agency. (Facility will evaluate each resident situation on a case-by-case basis.) However, state law requires any person providing "personal assistance in activities of daily living" to complete 40 hours of initial training. Of those 40 hours, 12 hours must be

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on dementia including the use and misuse of antipsychotics, the interaction of drugs commonly used by the elderly, the adverse effects of psychotropic drugs for use in controlling the behavior of persons with dementia, the special needs of persons with Alzheimer's disease and neurocognitive impairments, including nonpharmacologic, person-centered approaches to dementia care. Four hours of training must be on postural supports, hospice care and restricted health conditions. Sixteen hours must be "shadowing," or observed, hands-on training when delivering care. The remaining 8 hours of training must include

1. Physical limitations and needs of the elderly.
2. Importance and techniques for personal care services.
3. Residents' rights.
4. Policies and procedures regarding medications.
5. Psychosocial needs of the elderly.
6. Building and fire safety and the appropriate response to emergencies.
7. Cultural competency and sensitivity in issues relating to the underserved, aging, lesbian, gay, bisexual, and transgender community.

This training must be completed before private companion or caregiver delivering care. Proof of the training, done by a credible source, must be provided to facility. *Facility does NOT provide this training.*

Resident or resident's representative assumes full responsibility for the actions of a private companion. Resident or resident's representative are also responsible for paying the agency; or paying the individual minimum wage, payroll taxes, workers compensation, liability insurance, and a bonding fee. A worker's compensation certificate, proof of bonding and the private companion's legal right to work in the United States must be obtained and presented to facility prior to private companion being allowed in facility. Any fees incurred in obtaining the latter must be paid by resident or resident's representative. A private companion must wear a name tag, furnished by resident or resident's representative. (The content of the name tag will be provided by facility. The hiring of a private companion does not alleviate any additional levels of care fees as facility must provide all basic and optional services and must meet all resident service needs.

Any private companion found soliciting other residents for business, found to be on the premises without authorization, which "substitutes" for other private companions, will be barred from entering the property. A private companion is for resident only, and not allowed to switch to be a companion for any other facility resident.

\_\_\_\_\_ The private companion may not eat resident food but may opt to pay \$ 20 per meal if facility furnishes private companion's meals.

Resident or resident's representative is liable for any theft and loss that may be caused by the private companion.

Resident/Representative Initials \_\_\_\_\_ Facility Representative Initials \_\_\_\_\_

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### **Refund Conditions/Resident Death**

Under Health and Safety Code, 1569.652, states in part that an RCFE shall not “require advanced notice for terminating an admission agreement upon the death of a resident. No fees shall accrue once all personal property belonging to the deceased resident is removed from the living unit.” The facility will not, upon the death of a resident, “impede the removal of the resident’s personal property from the facility during reasonable hours” by persons authorized by the resident or the resident’s representative, to remove resident belongings.”

Facility will refund “any fees paid in advance covering the time after the resident’s personal property has been removed from the facility...within 15 days after the personal property is removed. If fees are assessed while a resident’s personal property remains in a unit after the resident is deceased, a licensee shall, within three days of becoming aware of the resident’s death, provide to the resident’s representative...written notice of the facility’s policies regarding contract termination upon death and refunds.”

In addition to the above stated law, facility will issue a refund if the Department orders the relocation of a resident and then, only, when advanced notice is required in an admission agreement [Title 22 section 87507(c)(5)(A)]. Facility’s admission agreement will not contain an advanced notice of intent to vacate, believing that would be a resident rights violation.

Facility is aware the resident or resident representative has purchased medication for resident use while residing in facility. Upon resident’s death, Federal Law prohibits facility from giving unused prescription medication to resident representative or others. Title 22 section 87465(i) requires unused prescription medications destroyed in the facility. However, unused over-the-counter medication can be returned to resident representative or others related to resident.

### **Belongings Removal**

In the event of resident’s death or in any way vacating of premises, facility will make reasonable efforts to assist resident and/or resident’s representative with belongings removal. Facility requires all resident personal belongings to be removed as quickly as resident’s representative can, but within five days of the resident’s death. This is not a legal requirement, but a facility provision under this contract. If not removed further charges may apply, prorated by a daily rate but not less than \$100 per day. Other arrangements may be made upon the agreement of both parties.

As mandated by Health and Safety Code 1569.153(f), if the resident’s representative is unable or unwilling to claim the property, facility must write the county’s Public Administrator and enclose the resident inventory of belongings. If the Public Administrator does not take possession of resident belongings, the facility may dispose of resident belongings unless alternative arrangements are made. Facility is not responsible for storing resident belongings. Signing below acknowledges the law and the facility’s policy about belongings removal.

Resident or Resident's Representative

### **Termination of Service Agreement/Resident Death**

The agreement is automatically terminated by the death of the resident. Because the facility accepts its fees at the first of each month, in the event of the resident's death, from that date, the resident's representative would not be liable for any payment as all service fees has been paid. However, if optional services were utilized during the days prior to resident's death, then resident's representative would be responsible for these additional charges.

### **Verbal/Written Notice of Intent to Vacate**

Facility does not require resident to provide any advance notice of intent to move or vacate the premises. However, should a resident with mental capacity, as documented by the resident's physician or resident's representative give a written or verbal notice of intent to move from the facility, facility reserves the right to move in another client to occupy current resident's room based solely upon that notice. If resident is mentally incapacitated any verbal notice will not be used as intent to vacate. (That authority will be reserved for resident's representative.) Any date stated in writing or verbally will be the date facility will anticipate the move of the resident from the facility that will include all belongings and personal items.

### **House Rules/General Facility Policies** (Please See Attached House Rules Addendum)

Violations such as those listed below will be grounds for eviction. The resident cannot do the following:

1. Verbal or physical abuse directed towards other residents or staff
2. Smoking in the facility is strictly forbidden due to the health and safety risks to other residents and to facility staff
3. Use of profanity or offensive language
4. Purposeful destruction of facility property
5. Violent behavior
6. Harming or threatening to harm oneself or others
7. Health conditions that require staffing levels not available in the facility
8. Refusal to practice general hygiene (bathing, oral care, clean clothes, etc.)
9. Refusal to store prescribed and over-the-counter medication, nutritional supplements, cleaning products and disinfectants in a secured and locked central location to prevent medication errors or accidental wrong usage. Resident has a right to privacy and facility respects that right. However, having medications in resident room without a physician's authorization may result in facility citations and fines by the Department.
10. Noncompliance with physician orders that would result in a threat to the safety or well-being of the general resident population, i.e. refusal to treat an infection.
11. Resident theft of facility or other resident belongings.
12. Possessing or threatening to use a firearm, sharp object or other items that may constitute a danger to staff and other residents.

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13. Residents have the right to consume alcohol as part of their personal choice. However, to ensure the safety and well-being of all residents, we kindly request that alcohol use be discussed with the resident's physician to evaluate any potential conflicts with prescribed medications or health conditions.
14. In shared rooms, the resident must turn off any audible devices, including the television, cell phone, tabloid devices, etc. to prevent disturbing other residents. In private rooms, any auditory device must be maintained at a sound level that will not disturb other residents.
15. The facility has opted out of the "end of life option act" (act) or the "death with dignity" law. The facility does not provide information about the act, and resident may seek this information from their physician. Any medication obtained by resident and/or resident's representative is not obtained or stored by facility. Facility staff is not allowed to assist in the administration of medication intended to end the resident's life.
16. The use of medical marijuana is allowed if smokeless.
17. The facility may admit individuals whose gender identity or sexual orientation differs from most of the resident population. The facility does not tolerate discrimination by residents against other residents as this may violate the State's protected civil rights that include sex, race, color, religion, national origin, marital status, registered domestic partner status, ancestry, actual or perceived sexual orientation, or actual or perceived gender identity.
18. Resident's bed, at resident option, may be pushed up against a wall in the room if the resident has a clear path and ability to get out of bed without obstruction.
19. Facility asks its residents and residents' visitors to avoid overt displays of affection in public and common areas of the facility. Facility requests any sexual activities take place in residents' rooms only. Residents in shared rooms must be respectful of a roommate if and when engaging in consensual sexual activities. Facility will attempt to assist with privacy in shared rooms if roommates are not consenting to sexual relations with each other.

The facility hereby believes these house rules/general policies are reasonable and necessary. The resident has the right under California State Health and Safety Code 1569.885 to suggest changes to these policies.

### **Procedure for Suggesting House Rule Changes**

All resident suggestions must be in writing and presented to the Administrator. The Administrator will review written suggestions within five working days of receipt. In considering policy changes Administrator will present suggested changes to the resident council to ensure no resident right is violated or resident safety is compromised if a change occurs. After the review of written suggestions, Administrator will render a decision in writing to the person who submitted the suggestion.

### **Bedrails and Hospital Beds**

The Department allows the use of bedrails but under very strict conditions. Thus, the facility must approve all beds equipped with bedrails prior to occupancy, especially "hospital beds," to ensure compliance with state laws and regulations and facility policies. The State allows "half bedrails," but only to assist the resident with mobility, i.e. turning, repositioning and transferring. Under state regulations, bedrails require a

physician's order specifically stating for use with mobility and the bedrails must be as close to the head of the bed as possible. Facility requires half rails be no less than 2-3 inches from the head of the bed. Full rails are not allowed unless the resident is admitted onto hospice. Facility must approve any beds supplied by the hospice agency. However, facility discourages the use of bedrails due to heightened awareness of injuries occurring throughout the U.S. in assisted living facilities. Facility will discuss the use of bedrails and alternatives with resident and resident representative prior to placement. Alternatives may include "halo rails" and/or "high – low beds."

### **Resident Council**

Two or more residents admitted into the facility may form a resident council with the assistance of the facility. The council is composed only of residents, but other may be invited to participate in council meetings. A purpose of the resident council is to make recommendations to the administrator on how to improve the quality of daily living and promote and protect residents' rights. Any recommendations presented in writing to the administrator will be acted upon in 14 calendar days or less. The council can meet independently with outside persons or staff, and to be informed of the right to participate in the regulatory inspection process. Other provisions of a resident council are posted in this facility.

### **Family Council**

When requested by a member of the resident's family or resident's representative, facility will allow the formation of a family council and allow the council to meet in the facility during mutually agreed upon hours and in a common meeting room in the facility. A family council is a meeting of family members, friends and others of at least two residents who may confer in private without the presence of facility staff. Facility will assist in the council meeting by posting notices of meetings, and minutes, information or newsletters produced by the council. Facility is unable to write the notices, write the minutes or prepare council-related information. Any concerns or recommendations submitted in writing to the administrator by the council will be acted upon within 14 days or less.

### **Grievance Procedure**

Facility must allow resident and/or resident's representative to file a grievance, in writing, with the Administrator regarding facility's general policies. The Administrator will review the grievance within five working days of its receipt with the resident council and render a written decision to the individual filing the grievance. The resident or resident's representative has the right to file a grievance with Community Care Licensing or the State Ombudsman, or both, to resolve any complaints regarding facility practices. The following information is also contained on the resident's (personal) rights form: The telephone number of Community Care Licensing is (844)538-8766. The Ombudsman's telephone number is (800)640-4661. In addition, a confidential complaint may be filed by calling (844) 538-8766.

Under California law, a licensee, or representative of the licensee, shall not discriminate or retaliate in any manner, including eviction or threat of eviction, against any person



receiving the services of the licensee's residential care facility for the elderly on the basis, or for the reason that a person has initiated or participated in the filing of a complaint, grievance, or a request for inspection with the Department pursuant to this chapter, or has initiated or participated in the filing of a complaint, grievance, or request for investigation with the appropriate local ombudsman, or with the state ombudsman.

The resident has the right to present grievances and recommend changes in policies, procedures, and services to the staff of the facility, the facility's management and governing authority, and to any other person without restraint, coercion, discrimination, reprisal, or other retaliatory actions. The licensee will take prompt actions to respond to residents' grievances.

### **Abuse Reporting**

The personal rights form provided to the resident and the resident's representative provides information about procedures for reporting known or suspected elder abuse, including the toll-free telephone number of the State Long-Term Care Ombudsman's CRISISline and the telephone number of the nearest approved organization for long-term care ombudsperson activities.

### **Licensing Reports**

Licensing reports are available for review at the facility, and copies of licensing reports and other documents pertaining to the facility are available from the district office of the Department.

### **Visitor Guidelines**

We welcome and encourage visits from family and friends, as they are an important part of our residents' well-being and happiness. To ensure a harmonious environment for all residents and staff, we kindly ask that visitors coordinate their visits during hours that respect the daily routines and rest times of the residents. If you are uncertain about a suitable visiting time, please feel free to discuss with the resident or our staff to ensure the visit is convenient and enjoyable for everyone. Visitors are asked to be dressed appropriately and to not smoke in facility. Facility prohibits visitors from bringing in cigarettes or cigars, lighters, matches, guns, knives or other sharp objects, medications or money that have not been approved by the administrator. Facility discourages, but cannot insist upon, family bringing in food that may be contrary to any physician-prescribed diet. Facility will periodically provide resident and resident's representative with notices of facility activities and events. The resident's representative is encouraged to participate. Facility may ask visitors to wear a mask or other personal protective equipment, present proof of being fully vaccinated or other Covid-19 screenings before entry. Facilities policies on Covid-19 visitations are available upon request.

### **Advertising Dementia Special Care Provisions**

This facility advertises its dementia special care, programming, and/or environment for persons with dementia. The resident or resident's representative are hereby informed that facility features are described in facility plan of operation and is available upon request. The facility does NOT have secured perimeters; thus Title 22 sections 87705(k) (l) do not apply.

**Consent for Hospice Care and Facility Retention**

Should I require hospice services, this agreement hereby acts as my request to receive hospice care and to be retained in the facility while receiving such care. I understand the facility may be required to deny this request if my condition has progressed to a condition not allowed in a residential care facility [Title 22, Section 87724(i)], if the facility's hospice waiver capacity is at a maximum, or if roommate, if any, withdraws consent to any shared living space [Title 22, Section 87633(h)(5)(A)].

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Resident or Resident's Representative**Roommate Consent for Hospice**

Should resident occupy a shared room and the roommate requires hospice care, by signing below, resident consents to allow hospice into the shared living space. This consent may be withdrawn verbally or in writing by resident and/or resident's representative.

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Resident or Resident's Representative**Theft and Loss Program**

This facility is required to notify residents and resident's representative upon admission of its written policies regarding the facility's theft and loss prevention program. The resident or resident's representative does have the option to refuse to disclose personal clothing items and not have these items inventoried. \_\_\_\_\_ I opt to have my clothing inventoried or \_\_\_\_\_ I opt not to have my clothing inventoried. However, California Health and Safety Code 1569.153(d) and Title 22 section 87218(a)(1) mandates that other personal property items be inventoried at the time of admission and as brought into the facility after admission.

**All items must be requested in writing to be added to or deleted from the Personal Property Inventory List**, as mandated by California State Health and Safety Code 1569.153(d). Resident's representative is asked to assist facility in marking all resident items for identification purposes. I have received a copy of this policy and procedure, which includes California State Health and Safety Codes 1569.152 - .154. Facility includes in its "Application Fee" a charge for this mandated inventory.

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Resident or Resident's Representative**Telecommunications Device Notification**

This facility is required by Title 22 section 87507(b) to provide the resident and his or her resident's representative with notification that the resident is entitled to special communication equipment from the telephone company. It is the resident's or resident's representative's responsibility to secure such equipment. I have received the Telecommunications Device Notification Form.

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\_\_\_\_\_  
 Resident or Resident's Representative

### **Resident/Personal Rights**

The resident and the resident's representative, if any, is entitled under California State law to receive a copy of the resident's personal rights pertaining to admission and retention in this facility. Under Health and Safety Code 1569.889, the personal rights form must contain the Ombudsman's phone number. The form, produced by the Department, is LIC613C-2, and is provided to residents and responsible parties at the time of admission. I have received a copy. Under Title 22, Division 6, Chapter 8, Section 87468.2(f), the Department of Social Services does not have the authority to waive the personal rights in this regulation section.

\_\_\_\_\_  
 Resident or Resident's Representative

### **LGBT Rights**

The State of California enacted Health and Safety Codes 1439.50 – 1439.54, known as the LGBT Bill of Rights. Under these laws, facility is required to have resident acknowledge receipt of the following nondiscrimination notice:

"This facility does not discriminate and does not permit discrimination, including, but not limited to, bullying, abuse, or harassment, on the basis of actual or perceived sexual orientation, gender identity, gender expression, or HIV status, or based on association with another individual on account of that individual's actual or perceived sexual orientation, gender identity, gender expression, or HIV status. You may file a complaint with the Office of the State Long-Term Care Ombudsman (800) 231-4024 if you believe that you have experienced this kind of discrimination."

Your signature below attests you have been advised and acknowledge receiving this nondiscrimination notice.

\_\_\_\_\_  
 Resident or Resident's Representative

### **Right to Privacy in Accommodations**

The statutory and regulatory resident and personal rights included with this agreement allow a resident a "reasonable level of personal privacy in accommodations" as outlined in Health and Safety Code 1569.269(a)(2). In contrast, the Department has a certain level of inspection authority found in Title 22 section 87755(a) which states, "Any duly authorized officer, employee or agent of the licensing agency may, upon proper identification and upon stating the purpose of his/her visit, enter and inspect the entire premises of any place providing services at any time, with or without advance notice." However, this regulation was written prior to and long before the implementation of the above noted Health and Safety Code.

Facility knows the Department can enter the facility and inspect it but under the new law may not enter resident rooms without resident consent as this would constitute a violation of legally enacted privacy rights. Facility further believes the Department may not open resident's private drawers and closets looking through personal belongings without the written or verbal—expressed—permission of resident or resident's representative as this inspection would violate the resident's right to personal privacy in his or her accommodations. If a resident lacks the cognitive ability to express consent, then the Department must seek the resident's representative's written or verbal consent. Thus, to protect the resident's legal right to personal privacy, please indicate preference of the Department search resident's personal belongings and living space. Either consent above may be withdrawn at any time in writing by resident or resident's representative. (See Attached Addendum "Privacy Lock & Key Policy")

☐ I authorize the Department to inspect my personal and private living space.

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Resident or Resident's Representative

☐ I do not authorize the Department to inspect my personal and private living space.

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Resident or Resident's Representative

### Medical Decisions/Emergency Provisions

This facility is required by Title 22 section 87469 to provide the resident and his or her resident's representative with the State's brochure, "Your Right to Make Decisions About Medical Treatment" (PUB325). I have received a copy.

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Resident or Resident's Representative

### Advance Directive Information

Facility advises resident and representative to have a completed Do-Not-Resuscitate (DNR) or Physician Orders for Life-Sustaining Treatment (POLST) on file with facility and obtained prior to or within two weeks of admission. To comply with state regulations, the facility must provide the resident or resident's representative with the following information contained in Title 22 section 87469 (b), (c), and (d):

**87469(b):** Residents shall be permitted to have a Request Regarding Resuscitative Measures form on file, which can include Physician Orders for Life Sustaining Treatment (POLST) and/or a Do-Not-Resuscitate (DNR) Form in their facility file.

**87469(c)** If a resident who has an advance directive and/or request regarding resuscitative measures form on file experiences a medical emergency, facility staff shall do one of the following: (1) Immediately telephone 9-1-1, present the advance directive

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and/or request regarding resuscitative measures form to the responding emergency medical personnel and identify the resident as the person to whom the order refers. (2) Immediately give the advance directive and/or request regarding resuscitative measures form to a physician, registered nurse or licensed vocational nurse if the physician or nurse is in the resident's presence at the time of the emergency and assumes responsibility. (3) Specifically for a terminally ill resident that is receiving hospice services and has completed an advance directive and/or request regarding resuscitative measures form pursuant to Health and Safety Code section 1569.73(c), and is experiencing a life-threatening emergency as displayed by symptoms of impending death that is directly related to the expected course of the resident's terminal illness, the facility may immediately notify the resident's hospice agency in lieu of calling emergency response (9-1-1). For emergencies not directly related to the expected course of the resident's terminal illness, the facility staff shall immediately telephone emergency response (9-1-1). (4) Facilities that employ health care providers, other than Home Health Agencies or Hospice Agencies, may comply with Health and Safety Code section 1569.74. (Facility does not have such policies.)

**87469(d)** After following the procedure in Section 87469(c)(1), (2), (3), or (4), as outlined above, facility staff shall notify the resident's hospice agency and Health Care Surrogate Decision Maker, if applicable.

### **Informing Emergency Responders**

By state laws and regulations, the facility cannot make any other arrangements regarding the calling of 911 in case of resident emergency. If the resident is receiving hospice services from a licensed hospice agency, facility is allowed under Health and Safety Code 1569.73(c) to call the hospice first, not 911, if the resident experiences a life-threatening emergency related to the resident's terminal illness.

Facility does not have a policy prohibiting any employee from voluntarily providing emergency medical services, including, but not limited to, CPR, in response to a medical emergency unless resident has expressed, in writing, a request regarding resuscitation, DNR or POLST form. In addition, facility may adopt or enforce a policy authorizing any employee trained in emergency services to provide those services to facility residents. Notwithstanding the above, any available facility employee may voluntarily provide emergency medical services if a trained and authorized employee is not immediately available or is otherwise unable or unwilling to provide emergency medical services.

### **Personal Protective Equipment (PPE)**

This facility is required to practice basic universal precautions as defined in Title 22 section 87101(u). This includes handwashing with soap and water, the wearing of non-sterile gloves, proper disposal of infectious materials and the cleaning with a bleach-based disinfectant. However, if ordered by the Center for Disease Control, World Health Organization, California Department of Public Health, Department of Social Services or other authoritative health organizations, facility may be forced to take more drastic measures to prevent the transmission of highly infectious diseases, i.e., COVID-19, HIV,

SARS, flu of all types, H1N1, etc. Then, facility may have to invest in specialty personal protective equipment that is not part of its basic universal precautions' protocols.

These can and may include the purchase of N95 facemasks (or similar), gowns, foot coverings or booties, high-alcohol hand sanitizers, and other items. Facility must pass on these added costs to residents and visitors and will do so at its cost. These will be itemized on resident statements.

### **Emergencies/Natural Disasters**

Facility is prepared for but hopes that it will not be affected by a natural or man-made disaster. Facility cannot control the occurrences of earthquakes, floods, fires or other natural or man-made disasters (including terrorist incidents). Should a disaster occur, facility will implement its emergency disaster plan filed with the Department at the time of licensure. Such a plan is available to residents and their responsible parties. Facility cannot guarantee emergency authorities can reach facility or that facility has any priority with emergency agencies in the event of any disaster or emergency. Facility's off-duty staff may not be able to reach the facility in case of an emergency or may be compelled to attend to their own family's needs. Facility's on-duty staff may also have to attend to family needs and may attempt to leave the facility. However, licensee will ensure all residents' needs are met and supervision will be present 24-hours-per-day.

In addition, facility cannot guarantee it can evacuate the clients if roads, emergency authorities or other conditions prevent it. Facility will make every attempt to comply with Health and Safety Code 1569.695 in contacting emergency agencies, families, hospice agencies (if necessary) and others to assist in evacuation should it become necessary to vacate its premises. Facility is required to be self-reliant for at least 72 hours but may not be able to sustain its operation if premises become uninhabitable or evacuation is ordered by emergency agencies.

Facility will seek the assistance of resident's representative to assist in relocating and/or transporting the resident to temporary housing. Again, it is hoped that no emergency or disaster would necessitate evacuation, but recent events in our nation and around the world make it necessary to be prepared for any known disasters. Facility may and will seek the help of resident's responsible parties to ensure the health and safety of the client in the event of an emergency.

Under Health and Safety Code 1569.695, revised for 2019 by the California State Legislature, facility is required to make every attempt and effort to communicate with resident's representative during any emergency. Please provide facility with the best possible way(s) to reach resident's representative during an emergency or disaster. This may include landline, cellphone, email, social media, text messaging, etc.:

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### **Emergency Room (ER)**

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If resident is transported to the emergency room by ambulance or other means for an incident or 9-1-1 call, it is the responsibility of the resident's representative to provide the ER staff with resident's personal information, insurance, POLST and/or DNR, and health condition. Upon consent, facility can provide some information for emergency responders (see "consent" in this agreement). Facility may be unable to go with the resident to the ER and sit and wait with the resident, and facility may be unable to transport resident back to the facility. Resident's representative is charged with arranging with the facility or others to return the resident safely back to the facility, which may include non-medical transport companies. Facility will work with the family and the resident to insure a safe return. When a resident is transported to the ER, this represents a "transfer of care," and the facility is not liable for any incidents that may occur to the resident at the time the client is transported away from the facility. Facility cannot provide an escort.

**Access to Records/DSS Authority**

The facility must allow the Department to review resident records kept by the facility. However, under H&S Code 1569.269(a)(3) resident must consent to the release of personal and private information and may legally refuse this inspection. Please indicate which resident right will be exercised. Either consent may be withdrawn at any time in writing by resident or resident's representative.

☐ I authorize the Department to inspect my personal records kept by the facility.

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Resident or Resident's Representative

☐ I do not authorize the Department to inspect my personal records kept by the facility.

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Resident or Resident's Representative**Sharing of Resident Information—Home Health and Hospice**

Should resident require any health care services provided by a home health or hospice agency, resident hereby agrees to release all written information provided by the home health or hospice agency to the facility to ensure that the resident's care needs can be met by the facility, and to prevent any prohibited conditions from developing.

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Resident or Resident's Representative**Continuation of Agreement**

If the resident is away from the facility for hospitalization, vacation, temporary placement outside this facility, etc., the monthly rate continues. The facility will hold the room up to the last day of a paid month unless the facility is notified in writing of the resident's discharge. Individuals other than the resident must have the legal right to discharge as determined by the facility upon seeing legal documentation. If the resident does not

return to the room after being away and by the next monthly fee due date, the facility will hold the room for seven days upon receipt of a week's fee prorated on a daily basis. The weekly prorated fee is \$ 1384.61. For each additional seven-day period, another week's fee must be received. Failure to pay this fee by the due date may result in involuntary discharge from the facility. Title 22 section 87224 requires a 30-day notice of eviction for non-payment of fees.

### **Eviction Procedures**

The facility may, upon 30-day written notice to the resident and the resident's representative, if any, evict the resident on the following grounds as outlined by state regulations:

1. Nonpayment of the rate for basic services within ten days of the due date.
2. Failure of the resident to comply with state or local law after receiving written notice of the alleged violation.
3. Failure of the resident to comply with general policies of the facility. Said general policies must be in writing, must be for the purpose of making it possible for residents to live together and must be made part of the admission agreement.
4. If, after admission, it is determined that the resident has a need not previously identified and a reappraisal has been conducted pursuant to Section 87463, and the licensee and the person who performs the reappraisal believe that the facility is not appropriate for the resident.

### **Three-day Eviction Notice**

Upon permission of the Department of Social Services, Community Care Licensing, a three (3) day notice of eviction may be granted for good cause. Copies of any notice of eviction will be sent to the resident, resident's representative, and any placement agency. Community Care Licensing will be advised of the eviction within five days of facility issuing the notice of eviction. California law requires specific information to be included within the notice of eviction (notice to quit), and facility will comply with all legal requirements.

### **Sixty-day Eviction Notice**

Facility will provide a 60-day notice to resident and resident's representative if an eviction or involuntary transfer is the result of forfeiture of the facility's license; change of use of facility; the licensee sells or otherwise transfers the facility or facility property, except when change of ownership applies to transferring of stock when the facility is owned by a corporation and when the transfer of stock does not constitute a majority change in ownership; the licensee surrenders the license to the department; or the licensee abandons the facility.

### **Facility Responsibilities under Forfeiture of License or Change in Use of Facility**

If the above, 60-day eviction does occur facility will take all reasonable steps to transfer affected residents safely and to minimize possible transfer trauma, and shall, at a minimum, do all of the following:

Resident/Representative Initials \_\_\_\_\_ Facility Representative Initials \_\_\_\_\_

02/06/2022

1. Prepare, for each resident, a relocation evaluation of the needs of that resident, which shall include both of the following:
  - a. Recommendations on the type of facility that would meet the needs of the resident based on the current service plan.
  - b. A list of facilities, within a 60-mile radius of the resident's current facility, that meet the resident's present needs.
2. Provide each resident or the resident's representative with a written notice no later than 60 days before the intended eviction. The notice shall include all of the following:
  - a. The reason for the eviction, with specific facts to permit a determination of the date, place, witnesses, and circumstances concerning the reasons.
  - b. A copy of the resident's current service plan.
  - c. The relocation evaluation.
  - d. A list of referral agencies.
  - e. The right of the resident or resident's representative to contact the department to investigate the reasons given for the eviction.
3. Discuss the relocation evaluation with the resident and his or her resident's representative within 30 days of issuing the notice of eviction.
4. Any refund of preadmission fees will be according to schedule found on pages 1 and 2 of this agreement.

**Facility Responsibilities under Closure or Change of Use**

The facility is responsible for the following should eviction take place under closure or change of use provisions:

1. Submit a written report of any eviction to the licensing agency within five days.
2. Upon issuing the written notice of eviction, a licensee shall not accept new residents or enter into new admission agreements.
3. Refund any monies under the preadmission agreement terms found on pages 1 and 2, including payment of said fees within 15 days of issuing the eviction notice. In lieu of the refund, the resident may request that the licensee provide a credit toward the resident's monthly fee obligation in an amount equal to the preadmission fee refund due.
4. If the resident gives notice five days before leaving the facility, the licensee shall refund to the resident or his or her legal representative a proportional per diem amount of any prepaid monthly fees at the time the resident leaves the facility, and the unit is vacated. Otherwise, the licensee shall pay the refund within seven days from the date that the resident leaves the facility, and the unit is vacated.
5. Within 10 days of all residents having left the facility, the licensee, based on information provided by the resident or resident's legal representative, shall submit a final list of names and new locations of all residents to the department and the local ombudsperson program.
6. If seven or more residents of a residential care facility for the elderly will be transferred because of the forfeiture of a license or change in the use of the facility, the licensee shall submit a proposed closure plan to the department for

approval. The department shall approve or disapprove the closure plan, and monitor its implementation, in accordance with the following requirements:

- a. Upon submission of the closure plan, the licensee shall be prohibited from accepting new residents and entering into new admission agreements for new residents.
- b. The closure plan shall meet the requirements described in subdivision (a) and describe the staff available to assist in the transfers. The department's review shall include a determination as to whether the licensee's closure plan contains a relocation evaluation for each resident.
- c. Within 15 working days of receipt, the department shall approve or disapprove the closure plan prepared pursuant to this subdivision, and, if the department approves the plan, it shall become effective upon the date the department grants its written approval of the plan.
- d. If the department disapproves a closure plan, the licensee may resubmit an amended plan, which the department shall promptly either approve or disapprove, within 10 working days of receipt by the department of the amended plan. If the department fails to approve a closure plan, it shall inform the licensee, in writing, of the reasons for the disapproval of the plan.
- e. If the department fails to act within 20 working days of receipt of the original or the amended closure plan, the plan, or amended plan, as the case may be, shall be deemed approved.
- f. Until the department has approved a licensee's closure plan, the facility shall not issue a notice of transfer or require any resident to transfer.
- g. Upon approval by the department, the licensee shall send a copy of the closure plan to the local ombudsperson program.
- h. If a licensee fails to comply with the requirements of subdivision (a), and if the director determines that it is necessary to protect the residents of a facility from physical or mental abuse, abandonment, or any other substantial threat to health or safety, the department shall take any necessary action to minimize trauma for the residents. The department shall contact any local agency that may have placement or advocacy responsibility for the residents and shall work with those agencies to locate alternative placement sites, contact relatives or other persons responsible for the care of these residents, provide onsite evaluation of the residents, and assist in the transfer of residents.
- i. A resident of a residential care facility for the elderly covered under this section, may bring a civil action against any person, firm, partnership, or corporation who owns, operates, establishes, manages, conducts, or maintains a residential care facility for the elderly who violates the rights of a resident, as set forth in this section. Any person, firm, partnership, or corporation, who owns, operates, establishes, manages, conducts, or maintains a residential care facility for the elderly who violates this section shall be responsible for the acts of the facility's employees and shall be liable for costs and attorney fees. Any such residential care facility for the elderly may also be enjoined from permitting the violation to continue. The

remedies specified in this section shall be in addition to any other remedy provided by law.

### **Relocation Assistance**

The facility may assist residents during any relocation process to find a suitable facility. It is the resident's or resident's representative's responsibility to ensure the new facility can care for the resident within the confines of its license. Assistance from the facility is to provide the resident with the phone number of a facility referral agency.

The resident or resident's representative may appeal any involuntary transfer, discharge or eviction as allowed by Title 22 section 87224(e) to Community Care Licensing, including the right to have to an interdisciplinary team review, and have that appeal investigated. Also, Title 22 section 87469(h) allows the resident or resident's representative to seek another remedy if to do so would not threaten the health, welfare and safety of the resident, or other residents, visitors and staff.

### **Department Relocation**

If a resident has a health condition which cannot be cared for within the limits of the license, requires inpatient care in a health facility, or has a health condition prohibited by Title 22 Sections 87455(c) or 87615, the Department may/shall order the licensee to relocate the resident. In cases where the Department determines that the resident is in imminent danger because of a health condition(s) which cannot be cared for in the facility or which requires inpatient care in a licensed health facility, the Department shall order the licensee to immediately relocate the resident.

### **Legal Documentation**

The facility must have copies of all legal documents, which allow someone other than the resident to make decisions regarding health and finances. These documents include, but are not limited to, Durable Powers of Attorney for Health Care, and Conservatorship.

### **Planned Absences**

The facility requests at least a 72-hours' notice of resident planned absences to allow facility to prepare medications or other items that may be required. Facility must report to the State resident death; serious injury; an incident that threatens the welfare, safety or health of the resident; or an unexplained absence of the resident. Please immediately report to the facility if any of the above occurs while resident is in the custody of family.

### **Falls**

There are many factors that can contribute to falls—medication, poor eyesight, illness, poor oxygenation, footwear, resident mobility, aging, etc. Many residents are admitted to residential care homes because of fall risk. The facility provides the highest levels of care but is unable to provide one-on-one attention. Even under higher supervision residents are at risk of falling. Facility asks that resident rooms not be furnished with throw rugs or extension cords, and any oxygen tubing kept to a manageable length.

### **Severability**

If one provision or paragraph of this agreement shall be invalidated in a legal proceeding, the remaining provisions or paragraphs shall remain in full force and effect.

### **Failure to Pay**

Facility reserves all rights to collect any unpaid balances using all possible means. The resident or resident's representative may be liable for all reasonable legal fees and costs of collection. Failure to pay also constitutes a breach of contract, and facility may seek in a court of equity payment under the terms of this contract.

### **Release of Liability When in Family Custody**

By signing this agreement, I absolve this facility of any liability that occurs when the resident is away from the facility for a visit, outing, doctor's or similar medical appointment, day care, vacation, or similar event which requires the resident to be in the custody or care of someone other than facility staff.

### **Video Cameras, Facility and Resident**

Facility does not consent to or allow the resident or resident's representative's use of hidden or exposed cameras or video equipment, including the concealed or revealed use of recording equipment in resident room or other areas of the facility. It is strictly prohibited under California and Federal privacy laws. However, facility may use cameras under the following guidance by Department of Social Services:

"'Informed Consent' is defined as 'a person's agreement, or that of their legally authorized representative, to allow something to happen, made with full knowledge of the risks involved and the alternatives.'"

"'Full Knowledge is defined as "having or showing awareness or understanding; well informed; deliberate; conscious.'"

By signing below, the resident and/or resident's representative acknowledges facility has obtained informed consent and full knowledge that it may utilize, for resident safety, audible, non-recording video monitors and/or non-audio video cameras in the resident living space, including resident rooms. Residents' rights allow a resident to be "granted a reasonable level of personal privacy in accommodations, medical treatment, personal care and assistance, visits, communications, telephone conversations, use of the Internet, and meetings of resident and family groups."

The resident and/or resident's representative hereby gives consent for the placement of safety monitors, including video surveillance, in the facility. If the resident and/or resident's representative does not wish to have any monitors in place, does not sign this informed consent statement, it is best to seek placement in a facility that does not use safety monitoring equipment. If the resident is in a shared room, facility cannot guarantee a complete absence of video monitoring. Thus, the facility will attempt to eliminate any monitoring of the resident who refuses to provide consent or revokes



consent later. However, if this consent is not signed, this facility may be inappropriate for the client and another facility may be better suited.

In addition, the facility uses monitoring in areas of the facility considered non-private such as outdoor space, kitchen, living room, etc. These areas will continue to use such equipment with or without resident consent. If a permanent video recording is made, only facility staff, the resident, or resident's representative may view the recordings. If the recording is made and kept, it will not be part of the resident records. However, all confidentiality rights of the resident will be observed.

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Resident or Resident's Representative

### Consent to Photograph

By signing below, I hereby authorize this facility to use my image (photo) for identification purposes only to ensure resident safety and well-being. This may include photographing portions of resident's body to document skin condition. Any photos of this nature will be kept in strictest confidence and not used commercially. In addition, I hereby (please check one)

☐ authorize this facility to use resident image (photo) for possible use in public forms of advertising including brochures or local advertising.

☐ do not authorize this facility to use resident image (photo) for possible use in public forms of advertising including brochures or local advertising.

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Resident or Resident's Representative

### Annual Assessments

It may be necessary for resident to be assessed by a healthcare professional at least once per year and more often if resident has had a significant change in resident's condition. This is required by Title 22, Division 6, Chapter 8, Section 87705, Care of Persons with Dementia. The Department of Social Services may also require an assessment by a healthcare professional under Title 22, Division 6, Chapter 8, Section 87458. The cost of such assessments are the responsibility of the resident or resident representative.

### Consent

The signature(s) below give consent to the facility to release to emergency response personnel the following documents: centrally stored medication record; insurance information, physician assessment/report, and, if applicable, a do not resuscitate (DNR) or Physician Order for Life-Sustaining Treatment (POLST) form. All other resident records are confidential and are not released to any party or agency without the express written consent of resident or resident's representative.

**Signatures**

I agree to the terms stated in this contract. Under California Health and Safety Code 1569.887 and Title 22 section 87507(e), the signatures below represent the resident's or resident's representative's acknowledgement of this agreement's terms and conditions. I have read and do understand all the preceding pages. My signature below acknowledges receiving a copy of this agreement that I agree to all the terms and conditions of the agreement and know that the facility has the original agreement on file.

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Resident

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Date

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Authorized Representative

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Date

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Administrator or Designee

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Date